

YSP IBUPROFEN

SUSPENSION 20mg/ml

Ingredient(s):

Each ml contains:

Ibuprofen 20mg
Sodium Benzoate 1.2mg (as preservative)

Pharmacology (Summary of Pharmacodynamics and Pharmacokinetics):

Pharmacodynamics:

Ibuprofen is a nonsteroidal analgesic, antipyretic and anti-inflammatory agent and is effective in the relief of inflammation, pain and fever. Nonsteroidal anti-inflammatory drugs inhibit the activity of the enzyme cyclo-oxygenase, resulting in decreased formation of precursors of prostaglandins and thromboxanes from arachidonic acid.

Pharmacokinetics:

Chemical group: A propionic acid derivative.

Half life – Elimination: 1.8 – 2 hours.

Onset of action: Pain: 0.5 hour.

Time to peak effect: Fever: 2 – 4 hours.

Duration of action -

Fever : 5mg/kg dose – 6 hours.

10mg/kg dose – 8 hours or more.

Pain : 4 – 6 hours.

Indication(s):

Relieve of fever, pain, muscle and joint inflammation.

Dosage and Administration:

Age	Dose	Frequency
10 - 12 years	15 ml	3 times daily
7 - 9 years	10 ml	3 times daily
4 - 6 years	7.5 ml	3 times daily
1 - 3 years	5 ml	3 times daily
6 - 12 months	2.5 ml	3 - 4 times daily
< 6 months	May be given on the advice of a doctor	

After assessing the risk/ benefit ratio in each individual patient, the lowest effective dose for the shortest possible duration should be used. Shake well before use.

Route of Administration:

For Oral Administration.

Contraindication(s):

Hypersensitivity to aspirin, ibuprofen or other nonsteroidal anti-inflammatory drugs. Patients with active or history of peptic ulceration. Should not be given to patients whom aspirin and other NSAIDs induce symptoms of asthma, rhinitis or urticaria.

Precaution(s) / Warning(s):

- Ibuprofen suspension should be given with care to the elderly, to patients with cardiovascular disease, and in liver or renal failure.
- Patients with congestive heart failure, cirrhosis, diuretic-induced volume depletion, or renal insufficiency require local synthesis of vasodilating prostaglandins to maintain renal perfusion, and therefore these patients are at greater risk of developing renal dysfunction due to NSAID-induced inhibition of renal prostaglandin synthesis.
- Ibuprofen suspension should be discontinued in patients who experience blurred or diminished vision, or changes in colour vision.
- Patients with collagen disease may be at increased risk of developing aseptic meningitis.
- Ibuprofen suspension should be given with care to patients who have asthma or have ever suffered from asthma. If the child's symptoms (eg. earache) persist for > 3 days, consult the doctor.
- Observational studies have indicated that non-selective NSAIDs may be associated with an increased risk of serious cardiovascular events, principally myocardial infarction, which may increase with dose or duration of use. Patients with cardiovascular disease or cardiovascular risk of an adverse cardiovascular event in patient taking NSAID, especially in those with cardiovascular risk factors, the lowest effective dose should be used for the shortest possible duration.
There is no consistent evidence that the concurrent use of aspirin mitigates the possible increased risk of serious cardiovascular thrombotic events associated with NSAID use.
- NSAIDs may lead to the onset of new hypertension or worsening the pre-existing hypertension and patients taking anti-hypertensive with NSAIDs may have an impaired anti-hypertensive response. Caution is advised when prescribing NSAIDs to patients with hypertension. Blood pressure should be monitored closely during initiation of NSAID treatment and at regular intervals thereafter.

8. Fluid retention and oedema have been observed in some patients taking NSAIDs, therefore caution is advised in patients with fluid retention or heart failure.
9. All NSAIDs can cause gastrointestinal discomfort and rarely serious, potentially fatal gastrointestinal effects such as ulcers, bleeding and perforation which may increase with dose or duration of use, but can occur at any time without warning. Caution is advised in patients with risk factors for gastrointestinal events e.g. the elderly, those with a history of serious gastrointestinal events, smoking and alcoholism. When gastrointestinal bleeding or ulceration occurs in patients receiving NSAIDs, the drug should be withdrawn immediately. Doctors should warn patients about signs and symptoms of serious gastrointestinal toxicity. The concurrent use of aspirin and NSAIDs also increases the risk of serious gastrointestinal adverse events.
10. NSAIDs may very rarely cause serious cutaneous adverse events such as exfoliative dermatitis, toxic epidermal necrolysis (TEN) and Stevens-Johnson Syndrome (SJS), which can be fatal and occur without warning. These serious adverse events are idiosyncratic and are independent of dose or duration of use. Patients should be advised of the signs and symptoms of serious skin reaction and to consult their doctor at the first appearance of a skin rash or any other sign of hypersensitivity.
11. This product contains aspartame, thus it is unsuitable for phenylketonurics.

Pregnancy and Lactation:

Whilst no teratogenic effects have been demonstrated in animal experiments, the use of ibuprofen during pregnancy should, if possible, be avoided. In the limited studies so far available, ibuprofen appears in the breast milk in very low concentration and is unlikely to affect the breastfed infant adversely.

WARNINGS

RISK OF GI ULCERATION, BLEEDING AND PERFORATION WITH NSAID

Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor GI problems (eg. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms.

Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious adverse events and other risk factors associated with peptic ulcer disease (eg. alcoholism, smoking, corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.

Drug Interactions:

In therapeutic doses, no evidence of clinically significant interactions with other commonly used drugs has so far been observed. However, caution should be exercised in patients receiving oral anticoagulants, thiazide diuretics, ACE inhibitors, β blockers, methotrexate, lithium, other pain relievers containing aspirin, ibuprofen or other NSAIDs.

Side Effect (s) / Adverse Reaction(s):

Adverse reactions are usually mild and transient: Stomach discomfort, nausea, stomach ulcer, rashes or worsening of asthma, liver and kidney problems, dizziness, headache and hearing disturbance.

Symptoms and Treatment for Overdosage, and Antidote(s):

Gastric lavage may be performed and if necessary, blood electrolytes corrected.

Storage Condition(s):

Keep in a tight container. Store at temperature below 30°C. Protect from light and moisture.

Shelf-Life:

3 years from date of manufacturer.

Packing(s):

An orange color suspension with orange flavour.

Plastic bottle of 60ml, 100ml and 120ml.

Plastic bottle of 4L (for export only).



Manufacturer and Product Registration Holder:
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